INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nab-paclitaxel · every 3 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00230
Version 7

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

Treatment stages

01 **Nab-paclitaxel · every 3 weeks**
21-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Nab-paclitaxel · every 3 weeks · National Cancer Control Programme (NCCP) 00230

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new cough or breathlessness, fever or sudden illness promptly. Tell staff immediately about an infusion reaction or pain at the infusion site. Report numbness, tingling or loss of daily function before the next dose.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Control advanced cancer and symptoms while balancing side effects and treatment burden.

CHEMOTHERAPY

Nab-paclitaxel

Albumin-bound paclitaxel · Abraxane



Disrupts cell division by stabilising microtubules

This formulation carries paclitaxel with albumin, a protein. It interferes with the structures cells use to divide. Its dosing and allergy precautions differ from ordinary paclitaxel.

Albumin-bound medicine → Microtubules held stable → Cell division disrupted

What to watch for

Report numbness, tingling, weakness or problems with daily tasks before the next dose. Tell staff about an infusion reaction; new cough or breathlessness needs prompt assessment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This albumin-bound paclitaxel schedule is used for metastatic breast cancer after first-line treatment failure when standard anthracycline-containing treatment is unsuitable. The source requires a response review after two cycles and discontinuation if there is no response.

Your treatment setting

Metastatic breast cancer after first-line failure; standard anthracycline therapy unsuitable

Your exact schedule

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

Alternatives

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Nab-paclitaxel · every 3 weeks · NCCP 00230

Treatment and review

Nab-paclitaxel · every 3 weeks

21-day view



Treatment · Day 1
Day 1

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE

Nab-paclitaxel · every 3 weeks

Repeat according to benefit and tolerance

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00230 v7 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Nab-paclitaxel · every 3 weeks: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00230	Baseline eligibility / tests	Day 1 infusion	Review response after two cycles	Repeat q21 days if benefiting / tolerating	Next treatment plan

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Blood / organ function NCCP 00230	full blood count (FBC), renal, liver baseline	Review results before dose	FBC / renal / liver every cycle	Confirm counts for each planned dose	Investigate persisting problems
Heart / nerves NCCP 00230	Cardiac function if history / prior anthracycline	Neuropathy and symptom review	Cardiac review when clinically indicated	Assess response after two cycles	Report persistent symptoms
Cancer review METASTATIC · MARKERS	Confirm setting	Agree review plan	Response / symptom review	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Metastatic breast cancer after first-line failure; standard anthracycline therapy unsuitable. FBC, kidney and liver tests before treatment and each cycle. Review response after two cycles; the source specifies stopping if there is no response.

Ask / act: Confirm eligibility, test results and medicine list.

2 Other checks

At the specified review points

Baseline cardiac assessment for a significant cardiac history or prior anthracycline exposure; further assessment as clinically indicated. Review neuropathy before doses.

Ask / act: Confirm who arranges and reviews each check.

3 Support and practical plan

At each treatment or phase change

Routine allergy premedicines are usually not required for this albumin-bound formulation. Low-risk antiemetic support is separate; previous reactions may change the plan.

Ask / act: Choose the matching support plan; confirm active medicines.

4 Response and follow-up

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / act: Review symptoms, results and patient priorities.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your treatment and access plan

STEP 1

Before treatment

FBC, kidney and liver tests before treatment and each cycle. Review response after two cycles; the source specifies stopping if there is no response.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Nab-paclitaxel is infused over 30 minutes. Full visit time includes preparation and review.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Routine allergy premedicines are usually not required for this albumin-bound formulation. Low-risk antiemetic support is separate; previous reactions may change the plan.

Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

Blood and organ checks

Other treatment-specific checks

Baseline cardiac assessment for a significant cardiac history or prior anthracycline exposure; further assessment as clinically indicated. Review neuropathy before doses.

Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.

Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Nab-paclitaxel · every 3 weeks · NCCP 00230

Nab-paclitaxel · every 3 weeks

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** intravenous (IV)
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Energy / appetite	 **Days 1–21** May vary and build across cycles
Numbness / tingling	 **Days 1–21** Can build across doses
Reaction / breathing symptoms	 **Days 1–21** Report promptly whenever they occur

Timing varies: Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

Treatment-day changes: A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Treatment markers show the planned days. Symptom bands show broad possibilities, not measured risk curves or guaranteed symptom-free days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00230 v7 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell needs prompt advice on any day, including planned treatment-free weeks.

Between-treatment notes

Day 1–21: Reference cycle

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

Support around the medicines

1 **Blood counts and infection**

Fever, shivers or sudden illness needs prompt advice on any day.

2 **Nerve symptoms**

Report numbness, tingling or weakness before the next dose.

3 **Support medicines**

Follow the prescription for the active medicine and treatment day.

4 **Review the plan**

Discuss symptoms, recovery and the next treatment decision with the team.

Questions worth asking

Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

Try explaining the plan back: Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

Nab-paclitaxel

- Numbness / tingling or weakness, sometimes lasting
- Low blood counts, infection and sepsis
- Hair loss, fatigue and muscle / joint aches
- Infusion reactions; uncommon serious lung inflammation

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nab-paclitaxel · every 3 weeks · each treatment day

Active medicines: Nab-paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- Routine allergy premedicines are usually not required for this albumin-bound formulation. Low-risk antiemetic support is separate; previous reactions may change the plan.
- Nab-paclitaxel is an albumin-bound formulation. It is not interchangeable with ordinary paclitaxel or with the weekly nab-paclitaxel schedule.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk and the exact source; it is not automatically prescribed by this template.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00230 · VERSION 7

Nab-paclitaxel · every 3 weeks

Nab-paclitaxel on Day 1 every 21 days. Review response after two cycles; the NCCP protocol says to discontinue if there is no response. Further treatment depends on benefit and tolerance.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

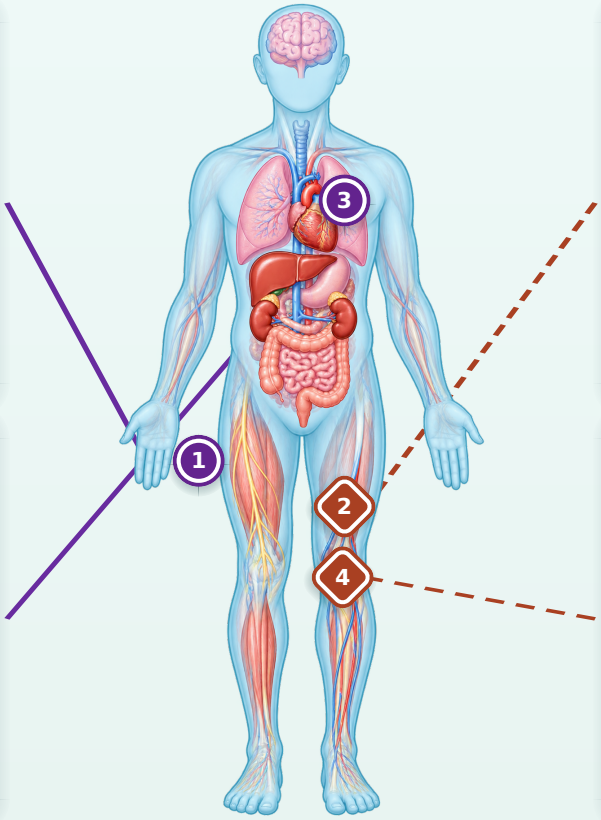
Nab-paclitaxel

Report numbness, tingling, weakness or loss of function.

2 Blood / infection

Nab-paclitaxel

Fever, shivers or suddenly unwell: call immediately.



3 Breathing / reaction

Nab-paclitaxel

Report new cough / breathlessness or an infusion reaction.

4 Muscles / joints

Nab-paclitaxel

Aches may follow treatment; report pain limiting daily tasks.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; this is not a complete list. Report new or persistent symptoms even after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

SACTCHECK · CHAPTER 04

Sources and further information

Nab-paclitaxel · every 3 weeks · National Cancer Control Programme (NCCP) 00230

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00230 v7 · official regimen**
   https://healthservice.hse.ie/documents/6565/230_v7_nab-PACLitaxel.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf